

Blood Pressure Value Edit Integrity Analysis

System, Specialty, Department, and Physician-Level Findings - Calendar Year 2025
Ambulatory Data Integrity & Quality Analysis | Spartanburg Regional Health System | July 14, 2026
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Executive Summary

This report analyzes the 20,357 blood pressure (BP) readings in the SRHS ambulatory EHR from CY25 that subsequently edited to a different value. The question is not whether individual edits are ever legitimate (they are - device re-checks, transcription fixes, and manual re-measurement all produce genuine corrections) but whether, in aggregate, the population of edits behaves like random correction noise or like a systematic, directional pattern. It does not behave like noise.

This is an analysis of the edit itself, not of BP control or of patient contacts generally. It covers only encounters where a recorded BP value was changed after entry; it does not include, and says nothing directly about, the - presumably much larger - population of BP readings entered during the same period that were never edited. Every percentage in this report describes the edited-encounter subset only.

Of 20,532 candidate edited-BP records, 20,357 (99.1%) passed data-quality screening (7 dropped for a missing edited value, 168 dropped as physiologically implausible). The analyzed set spans 271 physicians, 40 departments, and 2 specialties (Family Medicine, Internal Medicine). Across every level of aggregation tested - system, specialty, department, age cohort, and individual physician - the same four-part signature repeats: **edits are systematically lower than the original reading, edits compress the spread of values toward the center of the range, edits show a stronger preference for round numbers than the original readings did, and the pattern is more pronounced, not less, when the edit is made by someone other than the person who took the original reading.**

Headline Findings

- Systematic downward bias (H1): mean systolic change of -11.33 mmHg and mean diastolic change of -3.53 mmHg across all 20,357 edits, both significant at $p < 0.001$ (paired t-test and Wilcoxon signed-rank). Effect size for systolic is -0.60 (Cohen's d) - a medium-to-large effect, not a rounding artifact.
- Range compression (H2): the variance of edited systolic values is roughly half that of initial values (variance ratio $1.95\times$, $p < 0.001$); edited readings cluster into a narrower band (178-point range utilized vs. 229 initially).
- Round-number preference emerges upon editing, not before (H3): the terminal-digit chi-square test is not significant for either initial systolic or initial diastolic values, but is significant at $p < 0.001$ for both edited systolic and edited diastolic. Diastolic round-number share nearly jumps from 28.1% to 38.0% after editing.
- Clinical control-rate reclassification, within the edited subset: using the standard ≥ 140 systolic / ≥ 90 diastolic "uncontrolled" threshold used in CMS/NCQA hypertension-control quality measures, 78.2% of these edited encounters were initially uncontrolled; only 20.5% remain uncontrolled after editing. 58.6% of the 20,357 edited encounters - and 74.9% of the ones that started uncontrolled - were reclassified into the "controlled" category purely as a byproduct of the edit. This is not a system-wide control-rate estimate (Section 4); it describes only encounters that received an edit, a group that is plausibly weighted toward abnormal initial readings to begin with.
- The bias is stronger when the editor is not the original recorder: 68.3% of these edits (13,909 of 20,357) were made by someone other than the person who recorded the original value. This subgroup shows a larger effect size (-0.72 vs. -0.60 across the full edited set) and a much higher reclassification rate - 86.3% of initially-uncontrolled, different-user-edited encounters end up "controlled," compared with 47.7% for same-user edits (Mann-Whitney $U = 40,571,504$, $p < 0.001$).

1. Purpose and Scope

This analysis applies the same hypothesis-testing framework used in the preliminary half-year manual review (H1 2025, n = 10,019 initial readings) to the full CY25 population, now run as a reproducible notebook against the raw edit-history export rather than a manually built workbook. The underlying question is whether BP edits reflect legitimate clinical correction or a systematic pattern consistent with values being adjusted toward round, "cleaner," or better-controlled numbers after the fact. Because BP control rate is a component of multiple externally reported and reimbursement-linked quality measures (e.g., CMS MIPS BP control, NCQA Controlling High Blood Pressure), a directional edit pattern has downstream implications beyond documentation hygiene and it is reported here to surface that risk, not to assign individual intent; this analysis cannot distinguish a well-intentioned repeat measurement from a value adjusted to hit a target.

The analysis includes a dedicated sub-analysis isolating edits made by someone other than the clinician or staff member who recorded the original reading, since that pattern (one person's number being changed by a second person, often after the encounter) is a materially different workflow and risk profile than a single user correcting their own entry.

A scope note that applies to every finding in this report: the source data contains only encounters where a BP value was edited - it is not a sample of, or a stand-in for, all BP readings taken at SRHS in 2025, and edits are not made on anything close to 100% of patient contacts. If the decision to re-check and edit a value is itself triggered by an abnormal-looking first reading - a plausible, common workflow - then the edited population would look more "unwell" and more directional than the full patient population even before any bias in the edit is considered. This report should be read as a study of how edited values differ from their own originals, not as a statement about the BP status of SRHS's patient population as a whole.

2. Data and Methodology

2.1 Source and Screening

Source file: Edited BP Values, January 1 – December 31, 2025 (SRHS ambulatory EHR edit-history export). Each row represents one encounter where the recorded systolic/diastolic value was changed at least once after initial entry, with the initial and final (edited) values, the user who took/entered each, and the elapsed time between the original filing and the edit.

Screening Step	Rows
Rows in source file	20,532
Dropped - missing edited reading	7
Dropped - physiologically implausible value	168
Rows analyzed	20,357

Table 1. Data screening summary.

The analyzed population covers 271 unique physicians, 40 departments, and 2 specialties (Family Medicine and Internal Medicine - the only two specialties represented in this extract).

2.2 Hypotheses Tested

- H1 - Systematic Bias: paired t-test $p < 0.05$, $|\text{effect size}| > 0.20$, and mean difference $\neq 0$.
- H2 - Range Compression: F-test $p < 0.05$ and variance ratio > 1.50 , with the edited range narrower than the initial range.
- H3 - Digit Preference: round-number share $> 30\%$ and/or a significant terminal-digit chi-square test ($p < 0.05$).
- H4 - Department-Specific Bias: ANOVA across departments $p < 0.05$ and Levene's test $p < 0.05$ (variance itself differs by department).
- H4b - Age-Cohort-Specific Bias: same test structure as H4, applied across five age cohorts (<18 , $18-39$, $40-59$, $60-79$, $80+$, derived from single-year age at the time of the reading).

Each scope (system / specialty / department / physician / age cohort / editor) is scored against these criteria and assigned a Flag Level: CRITICAL (≥ 8 of 13 criteria met), HIGH (≥ 5), MODERATE (≥ 3), LOW (≥ 1), CLEAN (0), or INSUFFICIENT ($n < 10$, too small to test reliably).

2.3 Clinical Control-Rate Cross-Check

In addition to the notebook's statistical flagging criteria, this report includes a supplemental, independently computed metric: the share of encounters classified as "uncontrolled" (systolic ≥ 140 mmHg or diastolic ≥ 90 mmHg - the threshold used by CMS and NCQA hypertension-control quality measures) before versus after the edit. This metric is not one of the H1–H4b flagging criteria; it is included because it translates the statistical findings into a clinically and administratively legible quantity.

3. Results

3.1 System-Level Findings

"System-level" here means all 20,357 edited encounters pooled together, not all SRHS ambulatory encounters - see the scope note in Section 1. Within that edited population, all four hypotheses (H1, H2, H3, H4/H4b) are confirmed, and the pooled edited population flags CRITICAL (11 of 13 criteria met).

Metric	Value
Encounters analyzed	20,357
Mean systolic change (edited – initial)	–11.33 mmHg ($p < 0.001$)
Mean diastolic change (edited – initial)	–3.53 mmHg ($p < 0.001$)
Effect size, systolic / diastolic (Cohen's d)	–0.598 / –0.265
Variance ratio, systolic / diastolic (initial:edited)	1.95× / 1.67× ($p < 0.001$)
Value range utilized, initial → edited	229 → 178 points
Round-number share, all readings	28.7%
Terminal-digit chi-square	$\chi^2 = 6,146.8$, $p < 0.001$
Most common initial BP (count)	140/80 ($n = 319$)
Most common edited BP (count)	130/80 ($n = 577$)
ANOVA across departments (Systolic Δ)	$F = 70.69$, $p < 0.001$
Levene's test across departments (variance)	$F = 18.55$, $p < 0.001$
ANOVA across age cohorts (Systolic Δ)	$F = 79.82$, $p < 0.001$
Levene's test across age cohorts (variance)	$F = 30.52$, $p < 0.001$
Mann-Whitney, $ \Delta_{\text{systolic}} $ different- vs. same-user editor	$U = 40,571,504$, $p < 0.001$
Flag	CRITICAL (11/13)

Table 2. System-wide summary statistics, CY2025.

Two things stand out beyond statistical significance. First, the most common edited value (130/80, $n = 577$) is reached almost twice as often as the most common initial value (140/80, $n = 319$) - edits are not just moving values down, they are converging on a small set of "clean" numbers. Second, both the department-level and age-cohort-level Levene's tests are significant, meaning the degree of variability in editing behavior - not just the average - differs meaningfully by department and by patient age. Some departments and age groups compress values far more aggressively than others (Section 3.4, 3.8).

Figure 1. Distribution of Systolic Edit Size (n = 20,334)

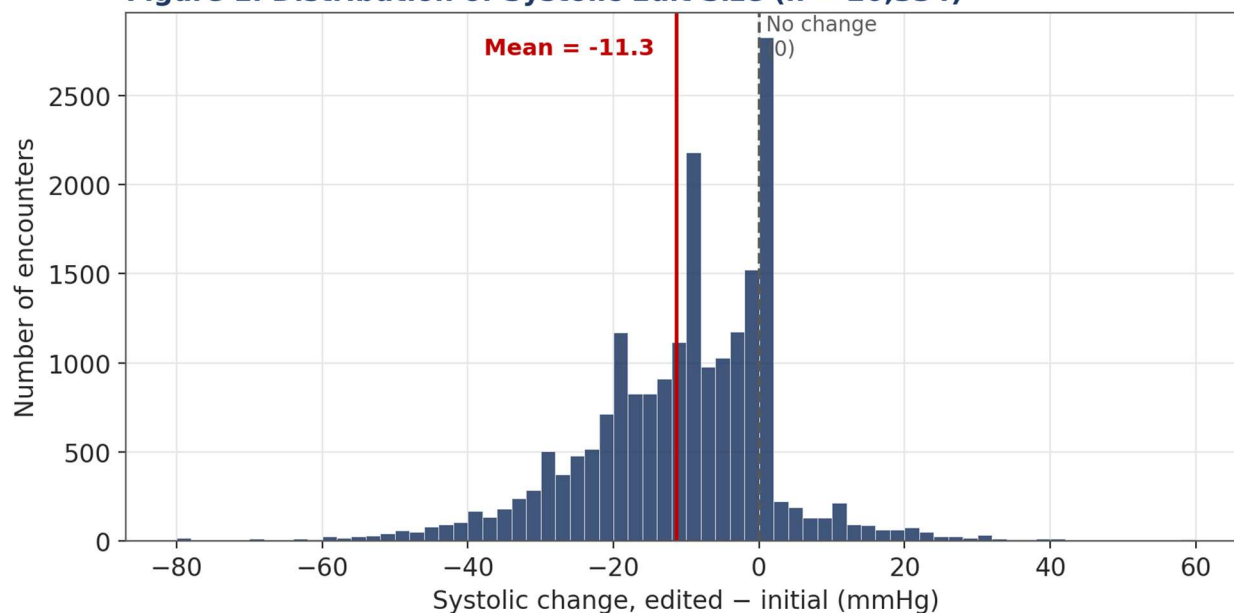


Figure 1. Distribution of the per-encounter systolic edit (edited – initial), all 20,334 plausibility-screened edited encounters. The distribution sits almost entirely left of zero and is not symmetric around it; the sharp spike at 0 partly reflects encounters where only the diastolic value was changed.

Figure 2. Initial vs. Edited Systolic Value Distributions

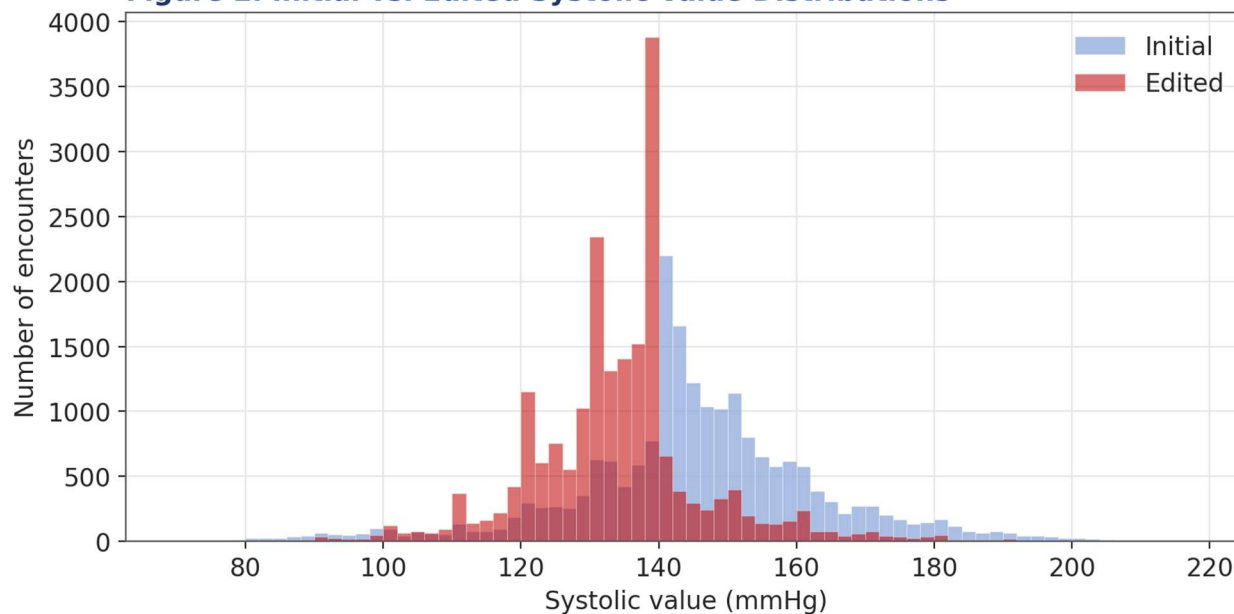


Figure 2. Initial (blue) vs. edited (red) systolic value distributions overlaid. The initial distribution is broad and centered in the mid-140s; the edited distribution is sharply peaked at 130 and 140 - visual evidence of both the downward shift (H1) and the range compression / round-number convergence (H2, H3).

3.2 Clinical Control-Rate Reclassification (Edited Subset Only)

Applying the $\geq 140/90$ "uncontrolled" threshold used in CMS/NCQA quality reporting to the initial and edited values, within the population of encounters that received a BP edit, shows a large shift in apparent BP control that is attributable entirely to the edit itself. This is not a system-wide BP control rate for SRHS - it describes only the 20,334 edited encounters in this dataset, and, per Section 1, that group is not known to be representative of all patient contacts:

Population	N	% Uncontrolled, Initial	% Uncontrolled, Edited	Reclassified to Controlled
All edited encounters	20,334	78.7%	20.5%	12,010 (59.1% of all; 75.1% of initially uncontrolled)
Edited by different user	13,902	81.6%	11.6%	9,792 (70.4% of subgroup; 86.3% of initially uncontrolled)
Edited by same user	6,432	72.3%	39.8%	2,218 (34.5% of subgroup; 47.7% of initially uncontrolled)

Table 3. Uncontrolled-BP reclassification, initial vs. edited value, split by editor type. (N reflects the plausibility-screened subset used for this cross-check, 20,334 of 20,357 rows - the edited-encounter population only, not all SRHS BP readings.)

In plain terms: among encounters that had a BP value edited, roughly 4 in 5 started in "uncontrolled" territory; after editing, only 1 in 5 remain there. Same-user edits still show a large shift, but different-user edits are nearly twice as likely to flip a patient's status from uncontrolled to controlled (86.3% vs. 47.7% of initially-uncontrolled cases). Whether this same reclassification effect exists among the much larger set of BP readings that were never edited cannot be determined from this dataset. Within the edited subset, though, this is the single most consequential finding in this report from a quality-measure integrity standpoint.

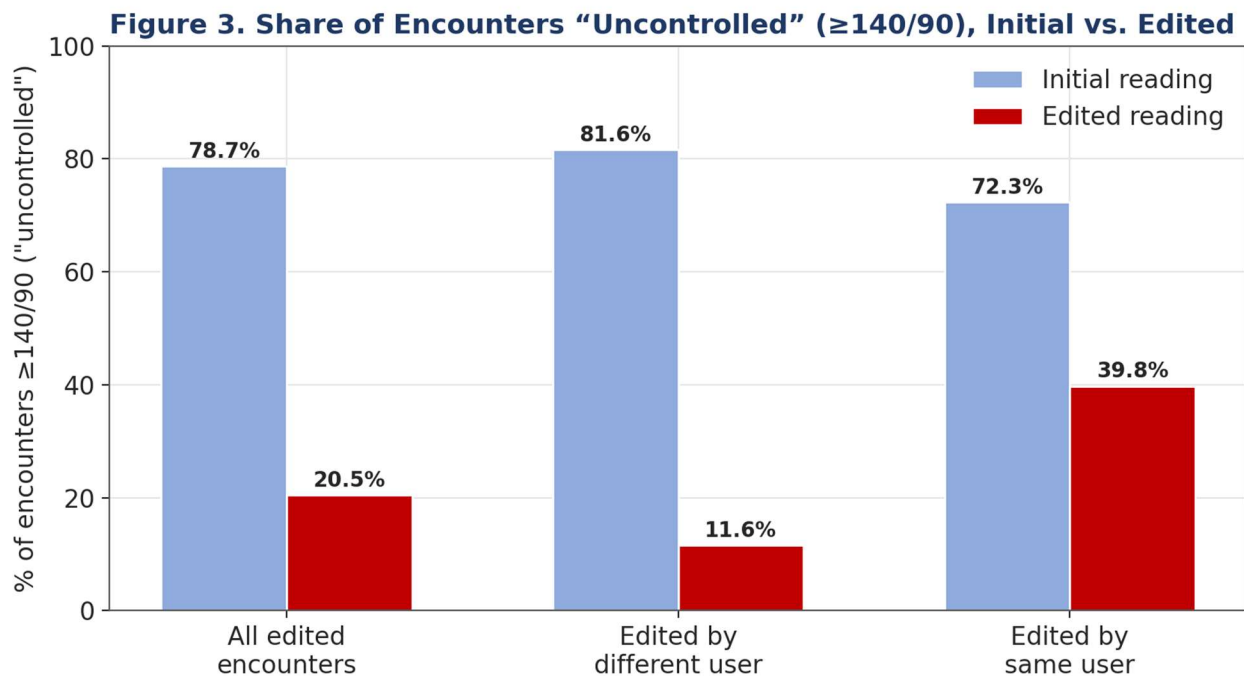


Figure 3. Share of edited encounters classified "uncontrolled" (≥ 140 systolic or ≥ 90 diastolic), initial vs. edited reading, for all edited encounters and split by editor type. Each pair is effectively a before/after proportion (a binomial share of encounters crossing the control threshold) - the different-user group shows the largest before/after gap.

3.3 Specialty-Level Findings

Specialty	N	% of Total	Effect Size (Sys/Dia)	Mean Δ Sys / Dia	Round-Number %	Flag
Family Medicine	14,506	71.3%	-0.639 / -0.228	-12.49 / -3.19	27.1%	CRITICAL
Internal Medicine	5,851	28.7%	-0.519 / -0.382	-8.46 / -4.38	32.6%	CRITICAL

Table 4. Specialty-level summary. Both specialties flag CRITICAL; Family Medicine shows the larger systolic effect, Internal Medicine the larger diastolic effect and the higher round-number share.

Specialty-Level Findings — Family Medicine vs. Internal Medicine (Edited-Encounter Subset)

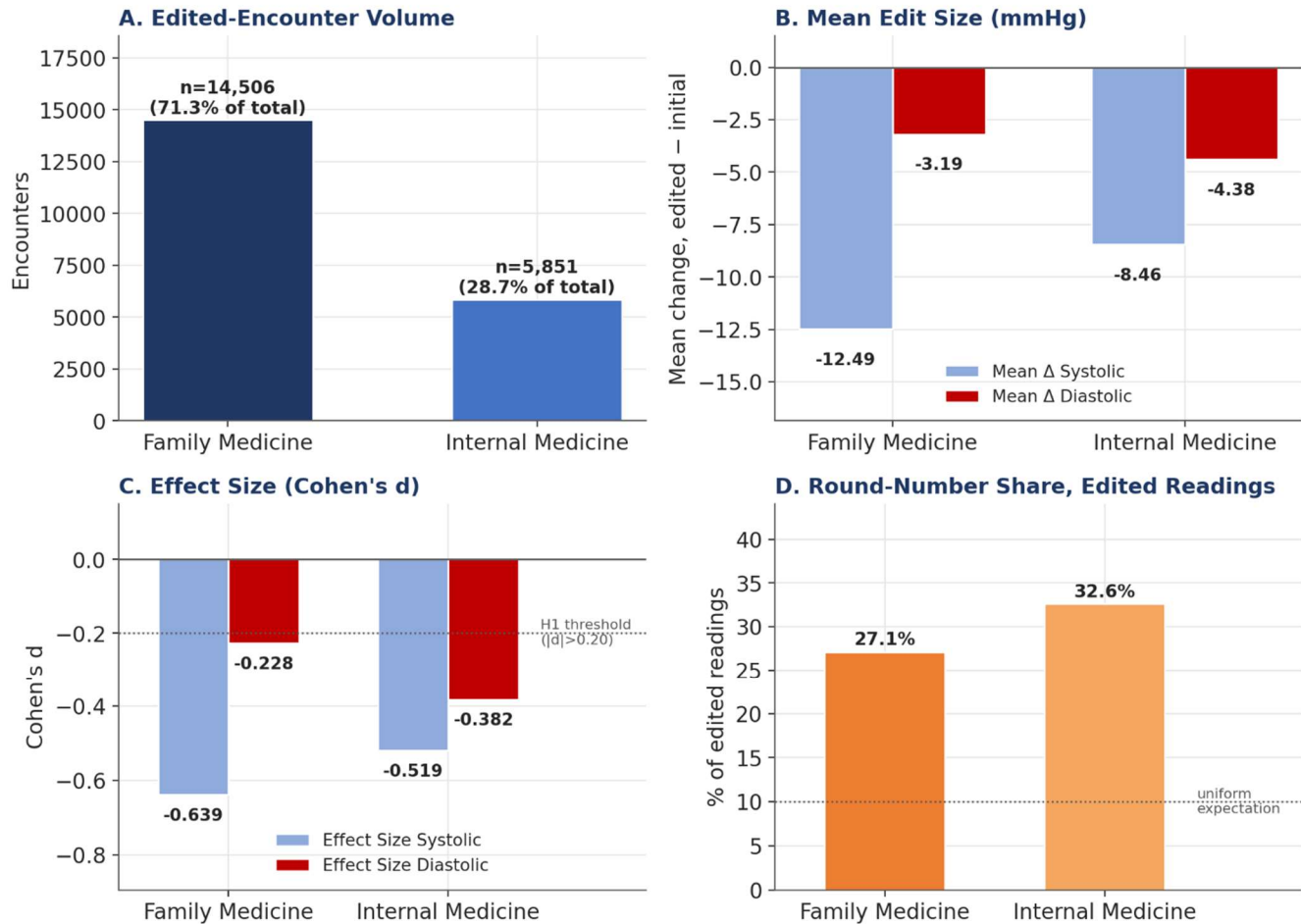


Figure 3.

Panel A shows edited-encounter volume by specialty; Family Medicine accounts for 71.3% of all edited encounters ($n = 14,506$) versus 28.7% for Internal Medicine ($n = 5,851$).

Panel B shows the mean systolic and diastolic edit size in mmHg - Family Medicine shows the larger systolic shift (-12.49 vs. -8.46), while Internal Medicine shows the larger diastolic shift (-4.38 vs. -3.19).

Panel C shows effect size (Cohen's d) for each measure against the H1 significance threshold ($|d| > 0.20$, dotted line); both specialties clear this threshold on both measures, with Family Medicine's systolic effect (-0.639) the largest of the four.

Panel D shows the round-number share of edited readings against the 10% uniform-digit baseline; Internal Medicine runs higher (32.6% vs. 27.1%), consistent with its larger diastolic round-number preference noted in Section 3.6. Both specialties flag CRITICAL overall (Table 4).

3.4 Department-Level Findings

34 of 40 departments have adequate volume ($n \geq 12$) to evaluate; the remaining 6 are small occupational-health clinics with 1–8 encounters and are marked INSUFFICIENT. Of the 34 evaluable departments, 25 flag CRITICAL, 8 flag HIGH, and 1 flags LOW - no department with meaningful volume comes back CLEAN. The three departments with the most criteria met (12 of 13) are MGC IM Eastside ($n = 1,017$), MGC FM Cedar Springs ($n = 757$), and the largest mean systolic shift of any department at -20.4 mmHg), and MGC FM Landrum ($n = 577$).

Department	N	Crit. Met	Flag	Mean Δ Sys	Mean Δ Dia	Round %
MGC IM EASTSIDE	1,017	12	CRITICAL	-7.86	-3.44	36.5%
MGC FM CEDAR SPRINGS	757	12	CRITICAL	-20.40	-5.24	23.0%
MGC FM LANDRUM	577	12	CRITICAL	-8.30	-3.40	45.8%
MGC FM CONVERSE HEIGHTS	2,349	11	CRITICAL	-17.44	-4.39	26.9%
MGC IM WESTSIDE	2,244	11	CRITICAL	-8.70	-7.08	29.0%
MGC FM DUNCAN	1,080	11	CRITICAL	-11.68	-4.89	28.1%
MGC FM WOODRUFF	552	11	CRITICAL	-13.41	-3.67	21.4%
MGC FM REIDVILLE RD	238	11	CRITICAL	-16.64	-4.36	23.5%
MGC FM FAM MED CTR	2,214	10	CRITICAL	-16.43	-1.89	23.7%
MGC MA IM NG	1,511	10	CRITICAL	-6.39	-2.70	35.1%
MGC FM INMAN	792	10	CRITICAL	-12.05	-1.60	28.5%
MGC ICC WESTSIDE	374	10	CRITICAL	-7.18	-3.39	21.4%
MGC FM GREER	346	10	CRITICAL	-6.51	-10.03	40.2%
MGC FM FOREST CITY	338	10	CRITICAL	-6.59	-2.50	34.3%
MGC ICC EASTSIDE	284	10	CRITICAL	-8.05	-4.52	24.3%
CMC CTR FOR FM GRSYPND	48	10	CRITICAL	-12.62	-4.21	27.1%
MGC IM SPTBG	737	9	CRITICAL	-15.13	-2.05	23.2%
UMC CTR FOR FM	506	9	CRITICAL	-10.37	-2.35	24.3%
MGC FM FIVE FORKS	478	9	CRITICAL	-6.30	-4.24	27.4%
MGC FM MOUNTAIN VIEW	342	9	CRITICAL	-7.95	-4.61	40.1%
CMC CTR FOR FM PEACHVW	1,017	8	CRITICAL	-12.18	-1.51	20.6%
MGC ICC REIDVILLE RD	172	8	CRITICAL	-8.23	-4.49	23.3%
CMC CTR FOR IM	86	8	CRITICAL	-5.91	-4.31	17.4%
MGC FM DORMAN	63	8	CRITICAL	-5.22	0.14	52.4%
MGC CAROLINA FAM PHY	531	7	HIGH	-8.51	-1.03	22.0%
CMC ICC	414	7	HIGH	-6.18	-1.10	28.5%
MGC FM PACOLET	234	7	HIGH	-13.55	-1.09	32.1%
MGC ICC PELHAM	169	7	HIGH	-10.21	-1.33	23.7%
MGC FM MILESTONE	165	7	HIGH	-4.04	-0.45	36.4%
SMC CTR FOR FM CHESNEE	36	7	HIGH	-7.69	-3.42	16.7%
MGC FM BOILING SPRINGS	275	6	HIGH	-2.67	-0.20	28.0%
MGC IM GREER	240	5	HIGH	-2.12	-0.91	67.5%
SMC CTR FOR FM	143	5	HIGH	-8.61	-2.43	25.2%
MGC SENIOR HLTH	12	2	LOW	0.58	3.00	50.0%

Table 5. All 34 evaluable departments, sorted by criteria met.

Figure 6. Mean Systolic Edit by Department (n ≥ 12)

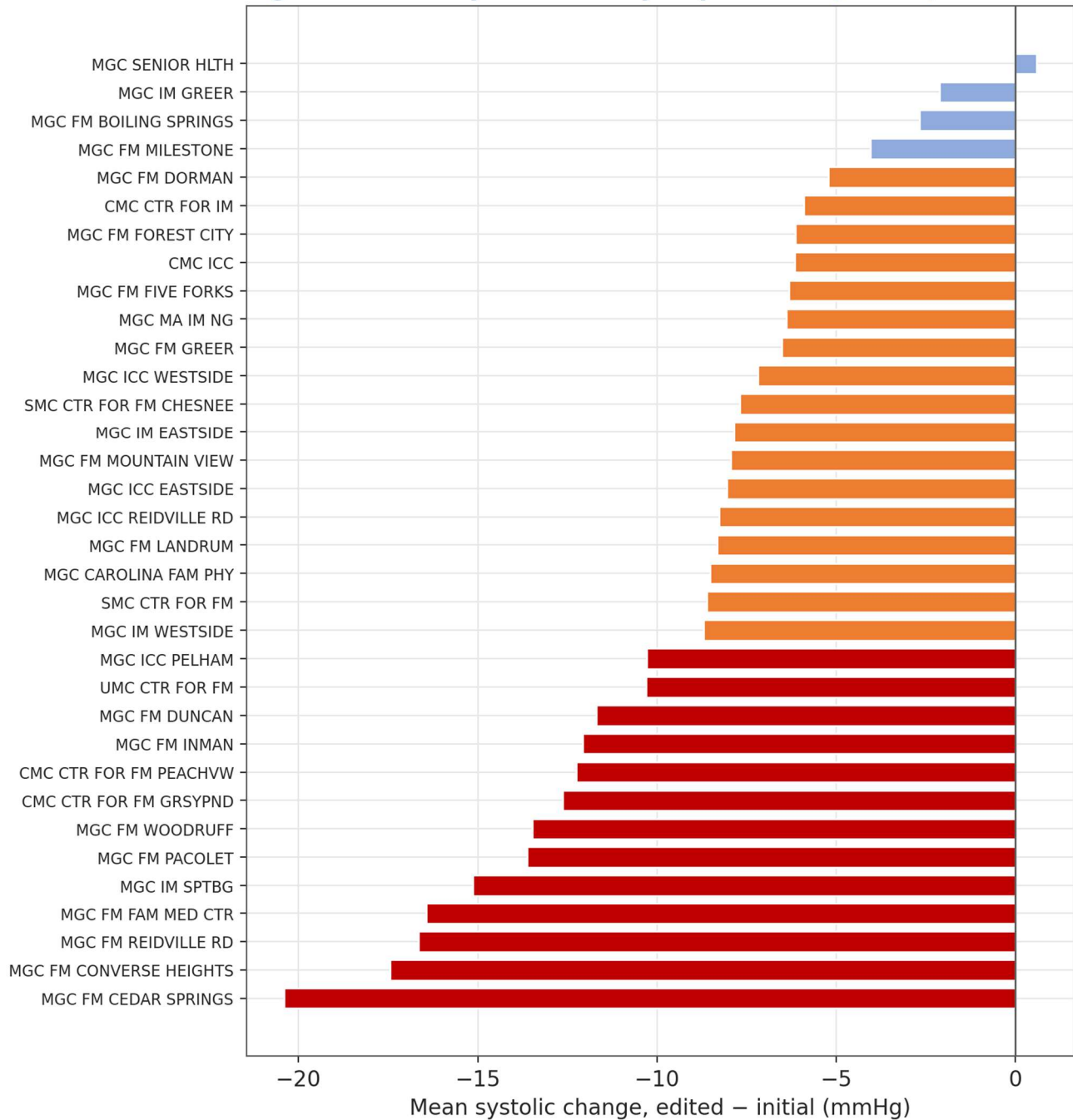


Figure 5. Mean systolic edit by department (n ≥ 12), sorted low to high. Red = CRITICAL-range shift (≤ -10 mmHg mean), orange = moderate shift (-5 to -10), blue = smaller shift. Every department with meaningful volume sits at or below zero. 6 additional departments (occupational-health clinics, n = 1–8) are excluded as INSUFFICIENT.

3.5 Physician-Level Findings

Of 271 physicians with at least one edited BP record, 107 have fewer than 10 encounters and are marked INSUFFICIENT. Among the remaining 164 physicians with adequate volume, 65 (39.6%) flag CRITICAL and 47 (28.7%) flag HIGH - together, 68.3% of evaluable physicians flag CRITICAL or HIGH. Only 6 physicians flag CLEAN.

Flag Level	Physicians	% of Evaluable (n=164)
CRITICAL	65	39.6%
HIGH	47	28.7%
MODERATE	26	15.9%
LOW	20	12.2%
CLEAN	6	3.7%
(INSUFFICIENT, n<10 - excluded above)	107	-

Table 6. Physician flag-level distribution.

The 20 physicians below (minimum n = 30) meet the most flagging criteria; several combine a high criteria count with substantial volume, meaning the pattern is not driven by small-sample noise for these individuals.

Physician (encounter provider)	N	Crit. Met	Flag	Mean Δ Systolic
CYKERT, DAVID MATTHEW	137	13	CRITICAL	-20.97
WALTER, CORNELIUS TODD	285	13	CRITICAL	-9.75
SOBOUTIPOUR, SHOHREH	105	12	CRITICAL	-21.02
MORROW, LORI ANNE	414	12	CRITICAL	-21.01
THOMPSON JR, JAMES CAROL	1,156	12	CRITICAL	-19.58
DORNA-PESQUERA, MANUEL ANTONIO	487	12	CRITICAL	-17.57
KUCABA, WALTER DEAN	290	12	CRITICAL	-13.97
TANBONLIONG, MICHAEL ONG	565	12	CRITICAL	-12.11
EVANS, HILARY JAMES	269	12	CRITICAL	-11.42
BRENNER, REBECCA	104	12	CRITICAL	-7.16
GABEL, DOROTEA MONY	113	11	CRITICAL	-25.73
REEL, TOD RAINSFORD	343	11	CRITICAL	-19.67
NOWATKA, THOMAS CHRISTIAN	170	11	CRITICAL	-17.50
GRANT, JAIMIE E.	59	11	CRITICAL	-17.20
BEARDEN, PATRICK	207	11	CRITICAL	-16.01
PERKINS, CASEY	232	11	CRITICAL	-15.81
GRAGG, JAMES MURIEL	439	11	CRITICAL	-13.80
KELLETT, NATALIE N	274	11	CRITICAL	-11.81
ARLAUSKAS, VICTORIA ANNE	1,761	11	CRITICAL	-8.34
SHUGART, HENRY EDWARD	302	11	CRITICAL	-6.96

Table 7. Top 20 physicians by criteria met (min. n = 30), by encounter provider of record.

3.6 Digit Preference (H3)

Round-number preference is markedly stronger after editing than before it, and the shift is larger for diastolic than systolic values - consistent with diastolic values being adjusted more freely (or to fewer distinct "clean" targets) than systolic.

Reading	N	χ^2 (terminal digit)	p	Round-Number %	Digit-Preference Flag
Initial Systolic	20,357	6,146.8	< 0.001	28.7%	No
Edited Systolic	20,357	11,575.3	< 0.001	30.2%	Yes
Initial Diastolic	20,357	5,992.8	< 0.001	28.1%	No
Edited Diastolic	20,357	16,591.2	< 0.001	38.0%	Yes

Table 8. Terminal-digit distribution test, initial vs. edited readings. The chi-square statistic is significant in all four rows (large N inflates significance for any deviation from uniform), so the Digit-Preference Flag column - based on the round-number-share threshold - is the more useful signal: it distinguishes initial (not flagged) from edited (flagged) values.

A secondary pattern in the terminal-digit distribution: edited systolic values show a pronounced excess of readings ending in 8 (23.0% of edited systolic values vs. 13.3% of initial), on top of the more familiar excess ending in 0 (24.6% vs. 22.6%). Edited diastolic values show the same secondary pattern (18.6% vs. 13.8% ending in 8). This is consistent with edits converging on a small set of memorable or "typical-looking" combined readings - 130/80, 138/80, 120/80 - rather than a pure rounding-to-nearest-10 effect.

Figure 4. Terminal-Digit Distribution, Initial vs. Edited Readings (dotted line = uniform 10%)

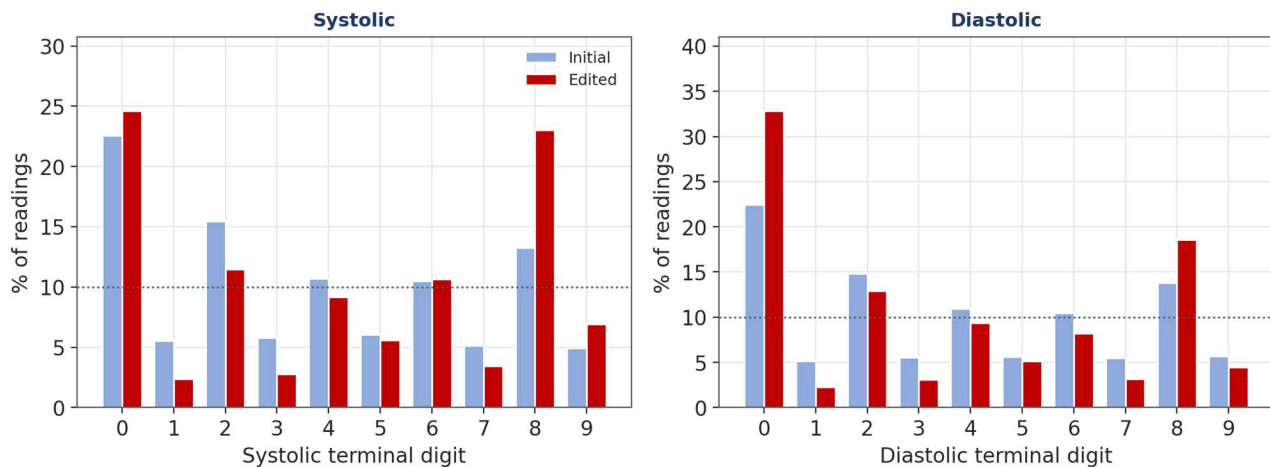


Figure 6. Terminal-digit distribution (last digit of each reading), initial vs. edited, systolic and diastolic. A uniform, unbiased distribution would sit at 10% for every digit (dotted line). Both edited distributions depart sharply from uniform at 0 and 8; the initial distributions depart less.

3.7 Sub-Analysis: Edits by a Different User

68.3% of all edits in this dataset (13,909 of 20,357) were made by a user other than the one who took and originally entered the reading. This is the largest single subgroup and, per Table 3, shows a meaningfully stronger bias and a much higher control-rate reclassification rate than same-user edits.

Scope	N	Effect Size (Sys/Dia)	Mean Δ Sys / Dia	Round-Number %	Flag
System (different-user edits)	13,909	-0.717 / -0.372	-12.22 / -4.49	29.6%	CRITICAL
Family Medicine (different-user)	8,812	-0.821 / -0.345	-14.18 / -4.29	28.5%	CRITICAL
Internal Medicine (different-user)	5,097	-0.567 / -0.435	-8.84 / -4.85	31.4%	CRITICAL

Table 9. Different-user edit subgroup, system and by specialty. Compare to Table 2 (system, all edits: effect size -0.598/-0.265) - the different-user subgroup shows a consistently larger effect at every level.

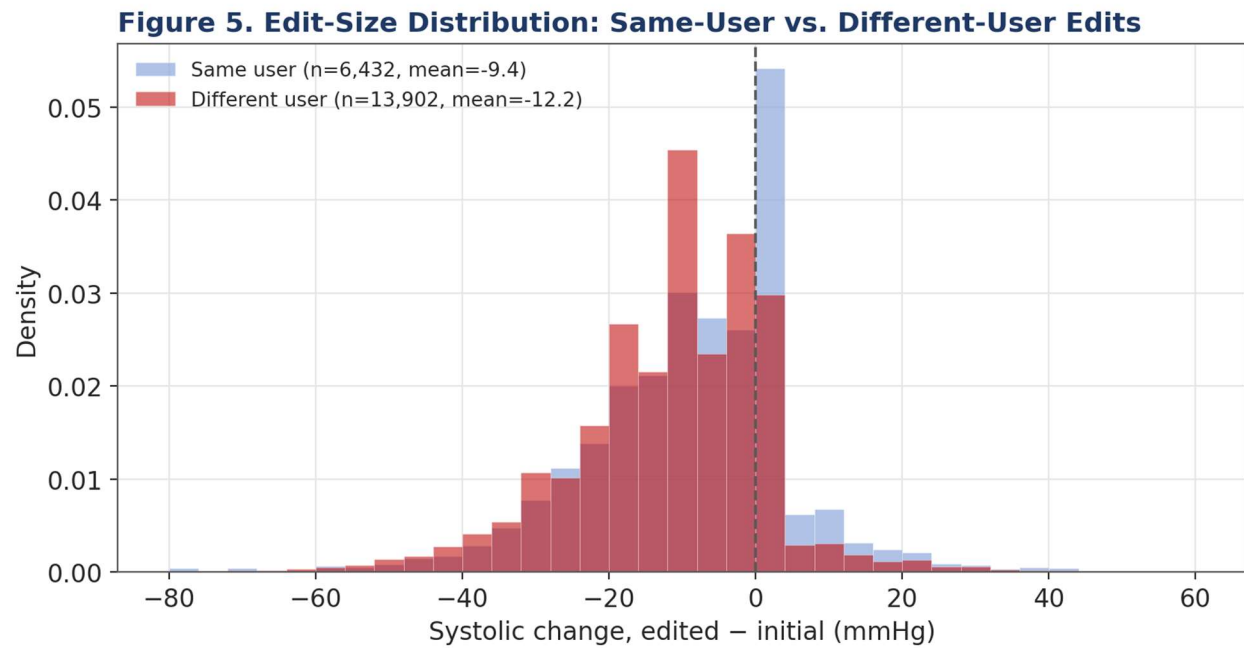


Figure 7. Distribution of per-encounter systolic edit size, same-user edits vs. different-user edits (density-normalized so the two groups are comparable despite different sample sizes). The different-user distribution sits further left, with a heavier and longer tail toward large downward edits.

Looking at who is doing the editing (rather than whose original reading is being changed), 298 distinct users appear as an "Edited_Entry_User" for at least one different-user edit. The 15 editors below (minimum n = 30) meet the most flagging criteria.

Editing User (Edited_Entry_User)	N	Crit. Met	Flag	Mean Δ Systolic
WALTER, CORNELIUS	269	13	CRITICAL	-10.07
CYKERT, DAVID M	130	13	CRITICAL	-22.14
THOMPSON, JAMES	1,122	12	CRITICAL	-20.10
TANBONLIONG, MICHAEL	543	12	CRITICAL	-12.42
DORNA-PESQUERA, MANUEL	463	12	CRITICAL	-18.31
MORROW, LORI	406	12	CRITICAL	-21.38
KUCABA, WALTER	260	12	CRITICAL	-15.18
EVANS, HILARY	237	12	CRITICAL	-12.62
BRENNER, REBECCA	90	12	CRITICAL	-8.09
SOBOUTIPOUR, SHOHREH	82	12	CRITICAL	-22.78
ZEPHIR, JORDAN	37	12	CRITICAL	-13.43
ARLAUSKAS, VICTORIA	1,724	11	CRITICAL	-8.49
KOVACS, SUZANNE	725	11	CRITICAL	-5.36
GRAGG, JAMES	335	11	CRITICAL	-12.76
REEL, TOD	329	11	CRITICAL	-20.24

Table 10. Top 15 editing users by criteria met (min. n = 30).

Notably, several names appear on both Table 7 (users whose recorded encounters show the pattern) and Table 10 (users who perform the edits) - e.g., Walter Cornelius Todd, Morrow Lori Anne, Tanbonliong Michael Ong, Dorna-Pesquera Manuel Antonio. This means the physician-level and editor-level flags are not fully independent signals and should be interpreted together, not summed.

The specific original-recorder → editor relationships with the most volume:

Original Recorder	Editing User	Edits	Mean Δ Sys	Mean Δ Dia	Round %
CANTRELL, JESSICA L	ARLAUSKAS, VICTORIA	543	−10.38	−8.96	16.8%
SANDERS, CRYSTAL	ARLAUSKAS, VICTORIA	509	−5.92	−8.69	15.7%
ROUSSEAU, SHERRY	TANBONLIONG, MICHAEL	477	−12.75	−6.56	22.9%
RUSSELL, GENEISE O	KOVACS, SUZANNE	466	−4.17	−3.82	42.9%
KENNEDY, MEGAN	BOLES, BAKER T	452	−17.51	−2.85	22.8%
EMORY, SYDNEE	THOMPSON, JAMES	445	−20.70	−5.91	38.4%
GOODE, AKIRIYA	THOMPSON, JAMES	444	−20.66	−5.68	19.1%
GOODWIN, NANCY J	FRIDAY, BEN	424	−13.58	−1.29	31.6%
MOSES, KETURAH	MORROW, LORI	337	−21.44	−4.91	22.6%
BRAGG, ALLIE M	REID, SAMUEL	317	−13.92	−1.58	23.3%

Table 11. Top 10 original-recorder → editor pairs by edit volume.

3.8 Age-Cohort Findings

Age Cohort	N	Crit. Met	Flag	Mean Δ Systolic
60–79	9,675	10	CRITICAL	−12.29
40–59	5,348	11	CRITICAL	−9.34
80+	3,324	11	CRITICAL	−13.48
18–39	1,805	10	CRITICAL	−8.63
<18	205	7	HIGH	−6.95

Table 12. Findings by age cohort. Every cohort flags HIGH or worse; the 80+ cohort shows the largest average downward shift.

4. Limitations

- This dataset covers only encounters where a BP value was edited (20,357 records); it excludes, and this report says nothing about, the presumably much larger population of BP readings entered in 2025 that were never edited. Edits are not made on anything close to 100% of patient contacts. Therefore, we cannot compute what fraction of all readings were edited, whether that fraction varies by department or provider, or whether the edited population is a random sample of all encounters. It most likely is not. Every statistic in this report describes the edited-encounter subset and should not be extrapolated to SRHS's patient population as a whole.
- Intent cannot be determined from this data. A systematic downward, round-number-converging edit pattern is consistent with target-driven adjustment but this analysis cannot distinguish that from other systemic causes (e.g., a standard workflow of re-measuring an elevated first reading and charting only the second, without a documented reason code) without chart-level review.
- Physiological-plausibility screening removed 168 of 20,532 rows using a fixed range filter; a small number of borderline-but-legitimate readings may have been excluded, and a small number of implausible-but-passing rows may remain.
- The clinical control-rate cross-check (Section 3.2, Table 3) uses a slightly different row count (20,334) than the main statistical analysis (20,357) because it applies its own independent plausibility bounds; the two are consistent in direction and magnitude but not derived from identical row sets.
- Physician- and editor-level flags are correlated, not independent (Section 3.7) - the same individuals frequently appear as both the recorder of flagged encounters and the editor of others' encounters.
- Neither the initial nor the edited value is validated against an independent, device-logged "true" reading; both are self-reported EHR entries, so this analysis measures the relationship between two recorded values, not accuracy against ground truth.
- Scope is limited to Family Medicine and Internal Medicine, the only two specialties present in this extract; findings should not be assumed to generalize to other specialties without a similar extract.
- Editor-pair figures (Table 11) with lower edit counts are more sensitive to a handful of encounters and should be treated as directional below roughly $n = 100$.

5. Conclusion

Every level of this analysis - system, specialty, department, physician, age cohort, and editor, all within the 20,357-record edited subset - points to the same pattern: BP edits in CY25 are systematically lower than the original reading, more tightly clustered around round or "clean" values than original readings are, and this pattern strengthens rather than weakens when the person editing the value is not the person who took it, overall consistent with the H1 CY25 analysis performed last summer. Within that edited subset, this pattern moves encounters from "uncontrolled" to "controlled" under the threshold used in externally reported BP-control quality measures - for 3 in 4 initially uncontrolled edited encounters, and closer to 6 in 7 when a different user makes the edit. This report cannot say what share of SRHS's total BP volume is edited, or whether the edited population's control rate mirrors the unedited population's; what it does establish is that the act of editing is not neutral within the population it touches, and it warrants review as a data-integrity and quality-measure-risk issue, independent of whether any individual edit was clinically appropriate.

The findings support three concrete next steps: (1) targeted chart-level audit of the highest-flagged physicians and editors in Tables 7, 10, and 11, where volume is large enough to rule out small-sample noise; (2) a workflow review of whether and how BP edits require a documented reason, since the current data contains no field capturing why a value was changed - that single addition would let a future run of this analysis separate legitimate correction from the pattern flagged here; and (3) pulling a denominator for total 2025 BP readings (edited and unedited) so a future version of this report can state what share of all patient contacts this pattern actually touches, rather than describing the edited subset alone.